Supplemental Data

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Appendix 1: Search terms

Intracranial pressure, intracranial hypertension, papilloedema, optical coherence tomography

1. Intracranial pressure
2. ICP
3. Intracerebral pressure
4. Intracranial hypertension
5. Cerebral swelling
6. Cerebral oedema
7. Cerebral edema
8. Brain swelling
9. Brain oedema
10. Brain edema
11. Papilloedema
12. Papilledema
13. Optical coherence tomography
14. OCT

Search combination:

1 OR 2 OR 3 OR 4 OR 5 OR 6 OR 7 OR 8 OR 9 OR 10 OR 11 OR 12

AND

13 or 14

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Appendix 2: Screening questions

Instructions for screeners: Tick the appropriate box per screening question. If "yes" at Stage 1, proceed to Stage 2; if "yes" at Stage 2, include. If "no" at any stage, exclude. If "unclear", proceed to next stage. If still "unclear" after Stage 2, then submit to third arbitrator for verdict.

Stage 1: Title Screening

Does the study represent Level 4 evidence or above, i.e. case series, case-control studies, cohort studies, randomised controlled trials (RCTs) and systematic reviews?

Yes	
No	
Unclear	

Does the study pertain to the use of optical coherence tomography (OCT) to detect raised intracranial pressure (ICP) in children, defined as 0 to 18 years of age?

Yes	
No	
Unclear	

Stage 2: Abstract screening

Does the study represent Level IV evidence or above, i.e. case series, cohort studies, case-control studies, randomised controlled trials (RCTs) and systematic reviews?

Yes	
No	
Unclear	

Does the study pertain to the use of optical coherence tomography (OCT) to detect raised intracranial pressure (ICP) in children, defined as 0 to 18 years of age?

Yes	
No	
Unclear	

Stage 3: Full paper screening

Does the study pertain to the use of optical coherence tomography (OCT) to detect raised intracranial pressure (ICP) in children, defined as 0 to 18 years of age?

Yes	
No	
Unclear	

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Appendix 3: Data extraction tool, adapted from the Cochrane Collaboration

General Information

Study title:
Author(s):
Source:
Date of Extraction:

Methods

Aim of study	
Study design	
Inclusion criteria	
Exclusion criteria	
Method of recruitment	
Method of randomisation (if applicable)	
Blinding (if applicable)	
Study duration	
Ethics approval	

Participants

Population description	
Setting	
Total number randomised	
Mean age	
Male/Female %	

Intervention 1

Group name	
Number in group	
Description	
Intervention parameter 1	
Intervention parameter 2 (if applicable)	
Intervention parameter 3 (if applicable)	
Intervention parameter 4 (if applicable)	
Intervention parameter 5 (if applicable)	
Intervention parameter 6 (if applicable)	
Intervention parameter 7 (if applicable)	
Intervention parameter 8 (if applicable)	
Drop-outs from group	

Outcome 1

Outcome group name	
Time points measured	

Outcome definition	
Outcome parameter 1	
Outcome parameter 2 (if applicable)	
Outcome parameter 3 (if applicable)	
Outcome parameter 4 (if applicable)	
Outcome parameter 5 (if applicable)	
Outcome parameter 6 (if applicable)	
Outcome parameter 7 (if applicable)	
Outcome parameter 8 (if applicable)	
Person measuring	
Imputation of missing data	

Intervention 2 (if applicable)

Intervention group name	
Number in group	
Description	
Intervention parameter 1	
Intervention parameter 2 (if applicable)	
Intervention parameter 3 (if applicable)	
Intervention parameter 4 (if applicable)	
Intervention parameter 5 (if applicable)	
Intervention parameter 6 (if applicable)	
Intervention parameter 7 (if applicable)	
Intervention parameter 8 (if applicable)	
Drop-outs from group	

Outcome 2 (if applicable)

Outcome group name	
Time points measured	
Outcome definition	
Outcome parameter 1	
Outcome parameter 2 (if applicable)	
Outcome parameter 3 (if applicable)	
Outcome parameter 4 (if applicable)	
Outcome parameter 5 (if applicable)	
Outcome parameter 6 (if applicable)	
Outcome parameter 7 (if applicable)	
Outcome parameter 8 (if applicable)	
Person measuring	
Imputation of missing data	

Intervention 3 (if applicable)

Intervention group name	
Number in group	
Description	
Intervention parameter 1	
Intervention parameter 2 (if applicable)	
Intervention parameter 3 (if applicable)	
Intervention parameter 4 (if applicable)	
Intervention parameter 5 (if applicable)	
Intervention parameter 6 (if applicable)	
Intervention parameter 7 (if applicable)	
Intervention parameter 8 (if applicable)	
Drop-outs from group	

Outcome 3 (if applicable)

Outcome group name	
Time points measured	
Outcome definition	
Outcome parameter 1	
Outcome parameter 2 (if applicable)	
Outcome parameter 3 (if applicable)	
Outcome parameter 4 (if applicable)	
Outcome parameter 5 (if applicable)	
Outcome parameter 6 (if applicable)	
Outcome parameter 7 (if applicable)	
Outcome parameter 8 (if applicable)	
Person measuring	
Imputation of missing data	

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Appendix 4: NIH Quality Assessment Tool for Observational Cohort and Cross-Sectional Studies

Criteria	Yes	No	Other (CD, NR, NA)*
1. Was the research question or objective in this paper clearly stated?			
2. Was the study population clearly specified and defined?			
3. Was the participation rate of eligible persons at least 50%?			
4. Were all the subjects selected or recruited from the same or similar populations (including the same time period)? Were inclusion and exclusion criteria for being in the study prespecified and applied uniformly to all participants?			
5. Was a sample size justification, power description, or variance and effect estimates provided?			
6. For the analyses in this paper, were the exposure(s) of interest measured prior to the outcome(s) being measured?			
7. Was the timeframe sufficient so that one could reasonably expect to see an association between exposure and outcome if it existed?			
8. For exposures that can vary in amount or level, did the study examine different levels of the exposure as related to the outcome (e.g., categories of exposure, or exposure measured as continuous variable)?			
9. Were the exposure measures (independent variables) clearly defined, valid, reliable, and implemented consistently across all study participants?			
10. Was the exposure(s) assessed more than once over time?			
11. Were the outcome measures (dependent variables) clearly defined, valid, reliable, and implemented consistently across all study participants?			
12. Were the outcome assessors blinded to the exposure status of participants?			
13. Was loss to follow-up after baseline 20% or less?			
14. Were key potential confounding variables measured and adjusted statistically for their impact on the relationship between exposure(s) and outcome(s)?			

*CD, cannot determine; NA, not applicable; NR, not reported

Quality Rating (Good, Fair, or Poor)	
Rater #1 initials:	
Rater #2 initials:	
Additional Comments (If POOR, please state why):	

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Supplemental Table 1: Excluded studies with reasons

Key: *Reasons for exclusion were as follows: A: adult studies (n=98); B: mixed adults and children without breakdown (n=27); C: conference abstracts (n=86); D: case reports (n=6); E: correspondence (n=4).

Number	Reference	Reason for exclusion*
1	Afonso, C.L., et al., Relationship between pattern electroretinogram, frequency-domain OCT, and automated perimetry in chronic papilledema from pseudotumor cerebri syndrome. Investigative Ophthalmology and Visual Science, 2015. 56(6): p. 3656-3665.	A
2	Afonso, J.M., et al., Spectral Domain-Optical Coherence Tomography As a New Diagnostic Marker for Idiopathic Normal Pressure Hydrocephalus. Frontiers in Neurology, 2017. 8: p. 10.	A
3	Aghsaei Fard, M., et al., Optic Nerve Head and Macular Optical Coherence Tomography Measurements in Papilledema Compared With Pseudopapilledema. Journal of neuro-ophthalmology : the official journal of the North American Neuro-Ophthalmology Society, 2019. 39(1): p. 28-34.	A
4	Albrecht, P., et al., Optical coherence tomography for the diagnosis and monitoring of idiopathic intracranial hypertension. Journal of neurology, 2017. 264(7): p. 1370-1380.	A
5	Alessi, G., et al., Evolution en tomographie par coherence optique de l'oedme papillaire dans l'hypertension intracranienne idiopathique traitee par mise en place de stent endosinusien du sinus lateralOptical coherence tomography in following up papilledema in idiopathic intracranial hypertension treated with lateral sinus stent placement. Journal Francais d'Ophtalmologie, 2010. 33(9): p. 637-648.	A
6	Anand, A., et al., Optical coherence tomography of the optic nerve head detects acute changes in intracranial pressure. Journal of Clinical Neuroscience, 2016. 29: p. 73-76.	A
7	Aojula, A., et al., Segmentation error in spectral domain optical coherence tomography measures of the retinal nerve fibre layer thickness in idiopathic intracranial hypertension. BMC ophthalmology, 2018. 17(1): p. 257.	A
8	Asian, F. and B. Ozkal, Optical coherence tomography measurements of the optic nerve head and retina in newly diagnosed idiopathic intracranial hypertension without loss of vision. Ceska a Slovenska Neurologie a Neurochirurgie, 2019. 82(3): p. 309-315.	A
9	Athappilly, G., et al., Ganglion Cell Complex Analysis as a Potential Indicator of Early Neuronal Loss in Idiopathic Intracranial Hypertension. Neuro-ophthalmology (Aeolus Press), 2019. 43(1): p. 10-17.	A
10	Auinger, P., et al., Baseline OCT Measurements in the Idiopathic Intracranial Hypertension Treatment Trial, Part II: Correlations and Relationship to Clinical Features. Investigative Ophthalmology & Visual Science, 2014. 55(12): p. 8173-8179.	A

11	Auinger, P., et al., Papilledema Outcomes from the Optical Coherence Tomography Substudy of the Idiopathic Intracranial Hypertension Treatment Trial. Ophthalmology, 2015. 122(9): p. 1939-1945.	A
12	Bassi, S.T. and K.P. Mohana, Optical coherence tomography in papilledema and pseudopapilledema with and without optic nerve head drusen. Indian journal of ophthalmology, 2014. 62(12): p. 1146-51.	A
13	Bilen, F.T. and H. Atilla, Peripapillary Vessel Density Measured by Optical Coherence Tomography Angiography in Idiopathic Intracranial Hypertension. Journal of Neuro-Ophthalmology, 2019. 39(3): p. 319-323.	A
14	Carey AR, Bosley TM, Miller NR, McCulley TJ, Henderson AD. Use of En Face Optical Coherence Tomography to Monitor Papilledema in Idiopathic Intracranial Hypertension: A Pilot Study [published online ahead of print, 2020 Mar 24]. J Neuroophthalmol. 2020;10.1097/WNO.0000000000000940. doi:10.1097/WNO.0000000000000940	A
15	Carta, A., et al., Optical coherence tomography is a useful tool in the differentiation between true edema and pseudoedema of the optic disc. PLoS ONE, 2018. 13(11).	A
16	Chen, J.J., et al., Optic disc edema in glial fibrillary acidic protein autoantibody-positive meningoencephalitis. Journal of Neuro-Ophthalmology, 2018. 38(3): p. 276-281.	A
17	Choi, S.S., et al., Changes in cellular structures revealed by ultra-high resolution retinal imaging in optic neuropathies. Investigative ophthalmology & visual science, 2008. 49(5): p. 2103-19.	A
18	Dreesbach, M., et al., Optic nerve head volumetry by optical coherence tomography in papilledema related to idiopathic intracranial hypertension. Translational Vision Science and Technology, 2020. 9(3).	A
19	Eren, Y., et al., Evaluation of optic nerve head changes with optic coherence tomography in patients with idiopathic intracranial hypertension. Acta Neurologica Belgica, 2019. 119(3): p. 351-357.	A
20	Fard, M.A., et al., Optic Nerve Head and Macular Optical Coherence Tomography Measurements in Papilledema Compared With Pseudopapilledema. Journal of Neuro-Ophthalmology, 2019. 39(1): p. 28-34.	A
21	Fard, M.A., et al., Optical Coherence Tomography Angiography in Optic Disc Swelling. American journal of ophthalmology, 2018. 191: p. 116-123.	A
22	Fard, M.A., et al., Optical Coherence Tomography Angiography in Papilledema Compared With Pseudopapilledema. Investigative ophthalmology & visual science, 2019. 60(1): p. 168-175.	A
23	Fard, M.A., et al., Quantification of peripapillary total retinal volume in pseudopapilledema and mild papilledema using spectral-domain optical coherence tomography. American journal of ophthalmology, 2014. 158(1): p. 136-43.	A
24	Flores-Rodríguez, P., P. Gili, and M.D. Martín-Ríos, Sensitivity and specificity of time-domain and spectral-domain optical coherence tomography in differentiating optic nerve head drusen and optic disc oedema. Ophthalmic Physiol Opt, 2012. 32(3): p. 213-21.	A
25	Gampa, A., et al., Quantitative Association Between Peripapillary Bruch's Membrane Shape and Intracranial Pressure. Investigative ophthalmology & visual science, 2017. 58(5): p. 2739-2745.	A

26	García-Montesinos, J., et al., Relationship between lamina cribrosa displacement and trans-laminar pressure difference in papilledema. Graefes Arch Clin Exp Ophthalmol, 2017. 255(6): p. 1237-1243.	A
27	Geddie, B.E., U.E. Altiparmak, and E.R. Eggenberger, Cup-to-disc ratio in patients with idiopathic intracranial hypertension is smaller than that in normal subjects. Journal of neuro-ophthalmology : the official journal of the North American Neuro-Ophthalmology Society, 2010. 30(3): p. 231-4.	A
28	Gozzi, F., et al., Optical coherence tomography in the differential diagnosis of true edema versus pseudoedema of the optic disc. Acta Ophthalmologica, 2017. 95.	A
29	Group, O.C.T.S.-S.C.f.N.I.I.H.S., et al., Baseline OCT measurements in the idiopathic intracranial hypertension treatment trial, part II: correlations and relationship to clinical features. Investigative ophthalmology & visual science, 2014. 55(12): p. 8173-9.	A
30	Hamill, E., et al., Cup-to-Disc Ratio in Idiopathic Intracranial Hypertension without Papilloedema. Neuro-ophthalmology (Aeolus Press), 2014. 38(2): p. 69-73.	A
31	Hata, M. and K. Miyamoto, Causes and Prognosis of Unilateral and Bilateral Optic Disc Swelling. Neuro-Ophthalmology, 2017. 41(4): p. 187-191.	A
32	Hata, M., et al., Measurement of retinal nerve fiber layer thickness in eyes with optic disc swelling by using scanning laser polarimetry and optical coherence tomography. Clinical ophthalmology (Auckland, N.Z.), 2014. 8: p. 105-11.	A
33	Hoye, V.J., 3rd, et al., Optical coherence tomography demonstrates subretinal macular edema from papilledema. Archives of ophthalmology (Chicago, Ill. : 1960), 2001. 119(9): p. 1287-90.	A
34	Huang-Link, Y., et al., Optical coherence tomography represents a sensitive and reliable tool for routine monitoring of idiopathic intracranial hypertension with and without papilledema. European journal of neurology, 2019. 26(5): p. 808-e57.	A
35	Huang-Link, Y.M., et al., OCT measurements of optic nerve head changes in idiopathic intracranial hypertension. Clinical Neurology and Neurosurgery, 2015. 130: p. 122-127.	A
36	Igarashi, N., et al., Optic disc cupping characteristics of normal pressure hydrocephalus patients with normal-tension glaucoma. Scientific reports, 2019. 9(1): p. 3108.	A
37	Intracranial, N.I., Baseline OCT Measurements in the Idiopathic Intracranial Hypertension Treatment Trial, Part I: Quality Control, Comparisons, and Variability (vol 55, pg 8180, 2014). Investigative Ophthalmology & Visual Science, 2016. 57(15): p. 6909-6909.	A
38	Johnson, L.N., et al., Differentiating optic disc edema from optic nerve head drusen on optical coherence tomography. Archives of ophthalmology (Chicago, Ill. : 1960), 2009. 127(1): p. 45-9.	A
39	Kamil, M.A.S., et al., Association between papilledema and guillian - Barre syndrome. Indian Journal of Public Health Research and Development, 2018. 9(12): p. 549-555.	A
40	Kaufhold, F., et al., Optic nerve head quantification in idiopathic intracranial hypertension by spectral domain OCT. PloS one, 2012. 7(5): p. e36965.	A

41	Kim, B.H. and E.J. Lee, Optic Disc Swelling After Intraocular Pressure Lowering Treatment in Acute Primary Angle Closure. Journal of glaucoma, 2017. 26(2): p. e87-e89.	A
42	Kim, M.J. and S.P. Kelly, Optical Coherence Tomography Imaging of Paton Folds in Papilledema With Retinopathy From Systemic Hypertension. JAMA ophthalmology, 2016. 134(10): p. e162121.	A
43	Koraysha, N.A., et al., Evaluating optic nerve diameter as a possible biomarker for disability in patients with multiple sclerosis. Neuropsychiatric Disease and Treatment, 2019. 15: p. 2571-2578.	A
44	Kupersmith, M., Baseline optical coherence tomography (OCT) measurements in the idiopathic intracranial hypertension treatment trial: Correlations and relationship to clinical features. Neurology, 2014. 82(10).	A
45	Kupersmith, M., Effects of intervention on optical imaging of papilledema in the idiopathic intracranial hypertension treatment trial. Neurology, 2015. 84.	A
46	Kupersmith, M., et al., Papilledema Outcomes from the Optical Coherence Tomography Substudy of the Idiopathic Intracranial Hypertension Treatment Trial. Ophthalmology, 2015. 122(9): p. 1939-1945.	A
47	Kupersmith, M.J. and N.I.S. Grp, Baseline Optical Coherence Tomography (OCT) of Participants in the Idiopathic Intracranial Hypertension Treatment Trial: Correlations and Relationships to Clinical Features. Investigative Ophthalmology & Visual Science, 2014. 55(13): p. 2.	A
48	Kupersmith, M.J., Baseline OCT measurements in the idiopathic intracranial hypertension treatment trial, part I: Quality control, comparisons, and variability. Investigative Ophthalmology and Visual Science, 2014. 55(12): p. 8180-8188.	A
49	Kupersmith, M.J., et al., Optical coherence tomography of the swollen optic nerve head: Deformation of the peripapillary retinal pigment epithelium layer in papilledema. Investigative Ophthalmology and Visual Science, 2011. 52(9): p. 6558-6564.	A
50	Kupersmith, M.J., et al., Papilledema Outcomes from the Optical Coherence Tomography Substudy of the Idiopathic Intracranial Hypertension Treatment Trial. Ophthalmology, 2015. 122(9): p. 1939-+.	A
51	Kupersmith, M.J., et al., Scanning laser polarimetry reveals status of RNFL integrity in eyes with optic nerve head swelling by OCT. Investigative ophthalmology & visual science, 2012. 53(4): p. 1962-70.	A
52	Kupersmith, M.J., Papilledema outcome evaluation in the OCT substudy of the idiopathic intracranial hypertension treatment trial. Investigative Ophthalmology and Visual Science, 2015. 56(7): p. 2235.	A
53	Labib, D.M. and D.H.A. Raouf, Diagnostic value of optical coherence tomography in patients with idiopathic intracranial hypertension. Egyptian Journal of Neurology Psychiatry and Neurosurgery, 2015. 52(4): p. 249-253.	A
54	Laemmer, R., et al., Detection of nerve fiber atrophy in apparently effectively treated papilledema in idiopathic intracranial hypertension. Graefes Archive for Clinical and Experimental Ophthalmology, 2010. 248(12): p. 1787-1793.	A
55	Lee, K.M., S.J. Woo, and J.-M. Hwang, Differentiation of optic nerve head drusen and optic disc edema with spectral-domain optical coherence tomography. Ophthalmology, 2011. 118(5): p. 971-7.	A

56	Liu, K.C., et al., Venous sinus stenting for reduction of intracranial pressure in IIH: A prospective pilot study. Journal of Neurosurgery, 2017. 127(5): p. 1126-1133.	A
57	Malhotra, K., et al., Association Between Peripapillary Bruch's Membrane Shape and Intracranial Pressure: Effect of Image Acquisition Pattern and Image Analysis Method, a Preliminary Study. Frontiers in neurology, 2018. 9: p. 1137.	A
58	Margolin, E., Diagnosis and grading of papilledema in patients with raised intracranial pressure using optical coherence tomography versus clinical expert assessment using a clinical staging scale. Evidence-Based Ophthalmology, 2011. 12(1): p. 42-43.	A
59	Marzoli, S.B., et al., Quantitative analysis of optic nerve damage in idiopathic intracranial hypertension (IIH) at diagnosis. Neurological sciences : official journal of the Italian Neurological Society and of the Italian Society of Clinical Neurophysiology, 2013. 34 Suppl 1: p. S143-5.	A
60	Monteiro, M.L. and C.L. Afonso, Macular thickness measurements with frequency domain-OCT for quantification of axonal loss in chronic papilledema from pseudotumor cerebri syndrome. Eye (Lond), 2014. 28(4): p. 390-8.	A
61	Oner, A., A. Agadayi, and N. Sinim, Optik Disk Odemi ve Optik Disk Druzeni Ayriminda Spektral-Domain Optik Kohorens Tomografi KullanilmasiDifferentiating optic disc edema from optic nerve head drusen with spectral domain optical coherence tomography. Retina-Vitreus, 2015. 23(2): p. 141-144.	A
62	Optical Coherence Tomography Substudy Committee; NORDIC Idiopathic Intracranial Hypertension Study Group. Papilledema Outcomes from the Optical Coherence Tomography Substudy of the Idiopathic Intracranial Hypertension Treatment Trial. Ophthalmology, 2015. 122(9): p. 1939-45.e2.	A
63	Optical Coherence Tomography Substudy, C. and N.I.I.H.S. Group, Papilledema Outcomes from the Optical Coherence Tomography Substudy of the Idiopathic Intracranial Hypertension Treatment Trial. Ophthalmology, 2015. 122(9): p. 1939-45.e2.	A
64	Pardon LP, Cheng H, Tang RA, Saenz R, Frishman LJ, Patel NB. Custom Optical Coherence Tomography Parameters for Distinguishing Papilledema from Pseudopapilledema. Optom Vis Sci. 2019;96(8):599-608. doi:10.1097/OPX.0000000000001408	A
65	Pardon, L.P., et al., Custom Optical Coherence Tomography Parameters for Distinguishing Papilledema from Pseudopapilledema. Optometry and vision science : official publication of the American Academy of Optometry, 2019. 96(8): p. 599-608.	A
66	Pasaoglu, I., et al., Lamina cribrosa surface position in idiopathic intracranial hypertension with swept-source optical coherence tomography. Indian journal of ophthalmology, 2019. 67(7): p. 1085-1088.	A
67	Patel, M.D., et al., Methods for Quantifying Optic Disc Volume and Peripapillary Deflection Volume Using Radial Optical Coherence Tomography Scans and Association With Intracranial Pressure. Frontiers in neurology, 2019. 10: p. 798.	A
68	Pilat, A.V., et al., Macular morphology in patients with optic nerve head drusen and optic disc edema. Ophthalmology, 2014. 121(2): p. 552-7.	A

69	Pilat, A.V., et al., Morphology of retinal vessels in patients with optic nerve head drusen and optic disc edema. Investigative ophthalmology & visual science, 2014. 55(6): p. 3484-90.	A
70	Rebolleda, G. and F.J. Muñoz-Negrete, Follow-up of mild papilledema in idiopathic intracranial hypertension with optical coherence tomography. Invest Ophthalmol Vis Sci, 2009. 50(11): p. 5197-200.	A
71	Rodriguez Torres Y, Lee P, Mihlstin M, Tomsak RL. Correlation Between Optic Disc Peripapillary Capillary Network and Papilledema Grading in Patients With Idiopathic Intracranial Hypertension: A Study of Optical Coherence Tomography Angiography [published online ahead of print, 2020 Jan 16]. J Neuroophthalmol. 2020;10.1097/WNO.0000000000000877. doi:10.1097/WNO.0000000000000877	A
72	Saenz, R., et al., Use of A-scan Ultrasound and Optical Coherence Tomography to Differentiate Papilledema From Pseudopapilledema. Optometry and vision science : official publication of the American Academy of Optometry, 2017. 94(12): p. 1081-1089.	A
73	Sarac, O., et al., Differentiation of optic disc edema from optic nerve head drusen with spectral-domain optical coherence tomography. Journal of neuro-ophthalmology : the official journal of the North American Neuro-Ophthalmology Society, 2012. 32(3): p. 207-11.	A
74	Savini, G., et al., Detection and quantification of retinal nerve fiber layer thickness in optic disc edema using stratus OCT. Archives of Ophthalmology, 2006. 124(8): p. 1111-1117.	A
75	Scott, C.J., et al., Diagnosis and grading of papilledema in patients with raised intracranial pressure using optical coherence tomography vs clinical expert assessment using a clinical staging scale. Archives of Ophthalmology, 2010. 128(6): p. 705-711.	A
76	Sheils, C.R., et al., The Relationship Between Optic Disc Volume, Area, and Frisen Score in Patients With Idiopathic Intracranial Hypertension. American journal of ophthalmology, 2018. 195: p. 101-109.	A
77	Sibony, P., et al., Optical coherence tomography shape analysis of the peripapillary retinal pigment epithelium layer in presumed optic nerve sheath meningiomas. Journal of neuro-ophthalmology : the official journal of the North American Neuro-Ophthalmology Society, 2014. 34(2): p. 130-6.	A
78	Sibony, P., M.J. Kupersmith, and F.J. Rohlf, Shape analysis of the peripapillary RPE layer in papilledema and ischemic optic neuropathy. Investigative ophthalmology & visual science, 2011. 52(11): p. 7987-95.	A
79	Sibony, P.A., et al., Retinal and Choroidal Folds in Papilledema. Investigative ophthalmology & visual science, 2015. 56(10): p. 5670-80.	A
80	Sibony, P.A., Gaze Evoked Deformations of the Peripapillary Retina in Papilledema and Ischemic Optic Neuropathy. Investigative ophthalmology & visual science, 2016. 57(11): p. 4979-4987.	A
81	Sinclair, A.J., et al., Low energy diet and intracranial pressure in women with idiopathic intracranial hypertension: prospective cohort study. BMJ (Clinical research ed.), 2010. 341: p. c2701.	A
82	Skau, M., et al., Disease activity in idiopathic intracranial hypertension: A 3-month follow-up study. Journal of Neurology, 2011. 258(2): p. 277-283.	A

83	Skau, M., et al., OCT for optic disc evaluation in idiopathic intracranial hypertension. Graefe's Archive for Clinical and Experimental Ophthalmology, 2011. 249(5): p. 723-730.	A
84	Smith, K.A., et al., A case series of dural venous sinus stenting in idiopathic intracranial hypertension: association of outcomes with optical coherence tomography¹. International Journal of Neuroscience, 2017. 127(2): p. 145-153.	A
85	Smith, S.V. and D.I. Friedman, The Idiopathic Intracranial Hypertension Treatment Trial: a Review of the Outcomes. Headache, 2017. (no pagination).	A
86	Starks, V., et al., Effect of optic nerve sheath fenestration for idiopathic intracranial hypertension on retinal nerve fiber layer thickness. Orbit (Amsterdam, Netherlands), 2016. 35(2): p. 87-90.	A
87	Steinegger, K., C. Bergin, and Y. Guex-Crosier, Malignant hypertension: clinical manifestations of 7 cases. Klinische Monatsblatter fur Augenheilkunde, 2015. 232(4): p. 590-2.	A
88	Tatar, I.T., et al., Morphological assessment of lamina cribrosa in idiopathic intracranial hypertension. Indian journal of ophthalmology, 2020. 68(1): p. 164-167.	A
89	Teismann, N., et al., Point-of-care ocular ultrasound to detect optic disc swelling. Academic Emergency Medicine, 2013. 20(9): p. 920-925.	A
90	Tirakotai, W., et al., Endoscopic Optic Nerve Sheath Fenestration for Treatment of Papilledema Secondary to Intracranial Venous Hypertension: Report of Two Cases. Journal of the Medical Association of Thailand = Chotmaihet thangphaet, 2016. 99 Suppl 3: p. S141-6.	A
91	Tsikata, E., et al., Volumetric Measurement of Optic Nerve Head Drusen Using Swept-Source Optical Coherence Tomography. Journal of Glaucoma, 2017. 26(9): p. 798-804.	A
92	Verma, R., et al., Ophthalmological manifestation in patients of tuberculous meningitis. QJM : monthly journal of the Association of Physicians, 2019. 112(6): p. 409-419.	A
93	Waisbourd, M., et al., OCT assessment of morphological changes of the optic nerve head and macula in idiopathic intracranial hypertension. Clinical neurology and neurosurgery, 2011. 113(10): p. 839-43.	A
94	Wall, M., et al., Threshold Static Automated Perimetry of the Full Visual Field in Idiopathic Intracranial Hypertension. Investigative Ophthalmology & Visual Science, 2019. 60(6): p. 1898-1905.	A
95	Wang, J.K., et al., Peripapillary retinal pigment epithelium layer shape changes from acetazolamide treatment in the idiopathic intracranial hypertension treatment trial. Investigative Ophthalmology and Visual Science, 2017. 58(5): p. 2554-2565.	A
96	Wang, J.K., et al., Three-dimensional bruch's membrane shape change over time with acetazolamide treatment in the idiopathic intracranial hypertension treatment trial (IIHTT). Investigative Ophthalmology and Visual Science, 2017. 58(8).	A
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